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Franciscan sisters' school

WHO committee: Research Report

**Addressing Global Healthcare Inequality Between Developed and
Underserved Communities.**



World Health Organization

Key Terms and Definition:

Health Equity: The state where everyone has a fair and just opportunity to attain their highest level of health, requiring the removal of systemic obstacles like poverty and discrimination.

Health Inequity: Avoidable, unjust, and unfair differences in health status, outcomes, or resource distribution between different population groups.

Social Determinants of Health (SDOH): The non-medical conditions in which people are born, grow, live, work, and ages such as income, education, and housing—that heavily influence health outcomes.

Universal Health Coverage (UHC): A healthcare system framework ensuring all people have access to needed promotive, preventive, curative, and rehabilitative health services without suffering financial hardship.

Primary Health Care (PHC): A community-based healthcare model that provides essential, affordable medical services close to where people live to prevent and treat the majority of their health needs throughout life.

Out-of-Pocket Healthcare Spending (OOP): Direct payments made by patients to medical providers at the time of service due to a lack of insurance or government coverage, which frequently pushes vulnerable families into poverty.

Financial Protection: A healthcare system safeguard that uses public funding or insurance to ensure people can access necessary medical treatment without suffering financial ruin or catastrophic debt.

Global Health Governance: The use of formal and informal institutions, rules, and processes by states and non-state actors to deal with challenges to health that require collective global action.

Health Literacy: The degree to which individuals have the ability to find, understand, and use information and services to inform health-related decisions and actions.

Brain Drain: The emigration of highly trained health professionals (such as doctors and nurses) from low-resource countries to wealthier nations, leaving underserved communities with severe staff shortages.

Background:

Over the past decades, major advancements in medicine and public health have improved health outcomes worldwide. However, these improvements have not been distributed equally, leaving

large gaps in access to healthcare among different communities. These gaps in healthcare access highlight the importance of health equity in global healthcare systems.

According to the World Health Organization, health equity refers to the absence of unfair and avoidable differences among groups of people, whether these differences are based on social, economic, geographical, or other factors. Health equity is also influenced by the conditions in which people are born, grow, live, work and age. However, achieving health equity remains difficult, as many communities continue to face barriers that limit their access to quality healthcare. These barriers can include poverty, high healthcare costs, geographical isolation, shortage of healthcare workers and limited medical infrastructure.

One major factor contributing to healthcare inequality is the unequal distribution of healthcare professionals. WHO estimates that the world could face a shortage of around 11 million health workers by 2030. This shortage can become even more severe due to “brain drain”, when trained healthcare workers leave their home countries or underserved regions in search of better salaries, working conditions, and opportunities elsewhere. As a result, communities that already have limited healthcare resources may be left with even fewer trained professionals, further widening the gap in access to quality healthcare.

Despite global progress, billions of people still struggle to receive the healthcare they need. According to WHO and the World Bank, around 4.6 billion people were not fully covered by essential health services in 2023, while approximately 2.1 billion people experienced financial hardship because of out-of-pocket healthcare spending in 2022. These numbers show that healthcare inequality remains a major global issue, not only in developing countries but also in developed countries, where low-income, rural, and other underserved communities can still face difficulties accessing quality healthcare. Therefore, addressing healthcare inequality requires attention not only to the availability of healthcare services, but also to their affordability, accessibility, quality, and financial protection across different communities.

Treaties and Organizations:

WHO - World Health Organization: The United Nations specialized agency responsible for international public health and for coordinating global solutions in order to combat various global public health issues worldwide.

ICESCR - International Covenant on Economic, Social and Cultural Rights (1966):
Adopted on 16 December, 1966. It recognizes the right of everyone to the highest attainable

standard of physical and mental health, making the right to health an important international legal commitment for countries that are parties to the Covenant.

Declaration of Alma-Ata (1978): Adopted at the International Conference on Primary Health Care (PHC) in 1978. It identified primary healthcare as a key method for achieving “Health for All” and emphasized that large health inequalities between countries are unacceptable and require international action.

WHO Pandemic Agreement (2025): Adopted on May 20, 2025, as a response to severe inequalities between communities in terms of accessing healthcare that were revealed during the COVID-19 pandemic. It aims to ensure equal access to healthcare and strengthen international cooperation.

WHO Global Code of Practice on the International Recruitment of Health Personnel (2010): Adopted by the World Health Assembly in 2010. It aims to promote ethical international recruitment of healthcare workers and reduce the negative effects that health-worker migration can have on countries already facing workforce shortages.

Sustainable Development Goal 3 – Good Health and Well-Being (2015): Adopted as part of the UN Sustainable Development Goals. SDG 3 aims to ensure healthy lives and promote well-being for all, while Target 3.8 specifically focuses on Universal Health Coverage, financial protection, and access to quality healthcare, medicines, and vaccines.

Current Situation:

Today, healthcare inequality continues to affect communities across the world despite the progress made toward Universal Health Coverage (UHC). According to WHO, the global UHC service coverage index increased from 54 in 2000 to 71 in 2023; however, progress has significantly slowed since 2015. This means that although more people are receiving essential healthcare services than in the past, large differences in accessibility, affordability, and quality of healthcare remain between and within countries.

Healthcare inequality can also be clearly seen within developed countries. For example, in the United Kingdom, a 2025 report by the UK Health Security Agency found that people living in the 20% most deprived areas of England were almost twice as likely to be admitted to hospital because of infectious diseases as those living in better areas. This shows that even countries with developed and funded healthcare systems can still experience major differences in health outcomes between wealthier and underserved communities.

In developing countries, limited healthcare infrastructure continues to create additional barriers. For example, in the community of Potali in India, residents previously had to walk many kilometers to receive basic medical treatment. A local healthcare center that had remained unused for approximately 20 years was reopened in January 2025 through an Indian government initiative with WHO support. The center recorded almost 2,500 outpatient visits during its first nine months, demonstrating the high demand for accessible healthcare in previously underserved communities.

Healthcare-worker shortages also remain a major concern. Since it also affects countries at different levels of economic development, especially in rural areas. For example, in 2025, Nigeria launched a national nursing and midwifery strategy for 2025–2030 to improve the deployment and performance of nurses and doctors and expand access to quality healthcare. Overall, healthcare inequality remains a global issue, as income, geographical location, healthcare costs, workforce shortages, and limited infrastructure continue to prevent many communities from receiving the care they need.

Critical Events and Timeline:

- **The Post-War Reconstruction era (1940s–1970s):** developed countries rapidly built modern hospital infrastructures and expanded government-provided health insurance while underserved areas were left to the incoherent remnants of colonial health systems and anemic missionary medicine.
- **The HIV/AIDS Crisis (1980s–1990s):** developed nations introduced Antiretroviral Therapy (ART) in 1996 which made HIV/AIDS a manageable chronic illness, but high

costs made ART unaffordable to underserved communities, where cheap generic drugs were unavailable, leading to millions of deaths.

- **Digital health and the Millennium Development Goals (2000s-2010s):** Electronic medical records, advanced diagnostic technologies and specialized medical care spread rapidly in industrialized countries. At the same time, there was a lack of clean water even in underserved areas, obstetric care was inadequate, and there was a shortage of doctors due to the exodus of highly qualified professionals.
- **Recovery from the pandemic and AI (2020s to present):** Industrialized countries are now using AI, mRNA technology and personalized precision medicine extensively. However, behind the scenes there is an extreme shortage of medical professionals, supply chains are disrupted, and the health impact of climate change are becoming increasingly difficult for underserved communities to manage.

Question to Consider:

1. How can the international community reduce disparities in healthcare access between developed and underserved communities, particularly in remote and low-resource areas?
2. What strategies can governments adopt to achieve Universal Health Coverage while ensuring affordable care and protecting vulnerable populations from catastrophic out-of-pocket spending?
3. How can countries address healthcare-worker shortages and brain drain while ensuring that underserved communities have a sustainable and adequately trained healthcare workforce?

4. What measures can countries take to improve the availability and affordability of essential medicines, vaccines, medical technologies, and digital healthcare services in underserved communities?
5. How can governments strengthen Primary Health Care and address social determinants so that poorer and disadvantaged communities have the same opportunity to stay healthy as everyone else?
6. How can international cooperation among governments, WHO, international organizations, and other relevant stakeholders strengthen healthcare systems and ensure that resources, knowledge, technology, and healthcare services reach underserved communities effectively?

Relevant Media:

Figure 1. Population reporting unmet needs for medical care, by main reason. (2024)

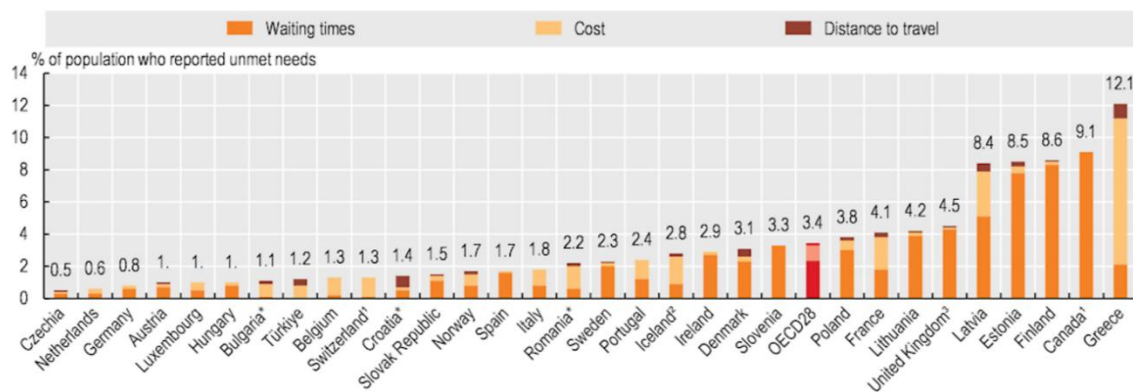
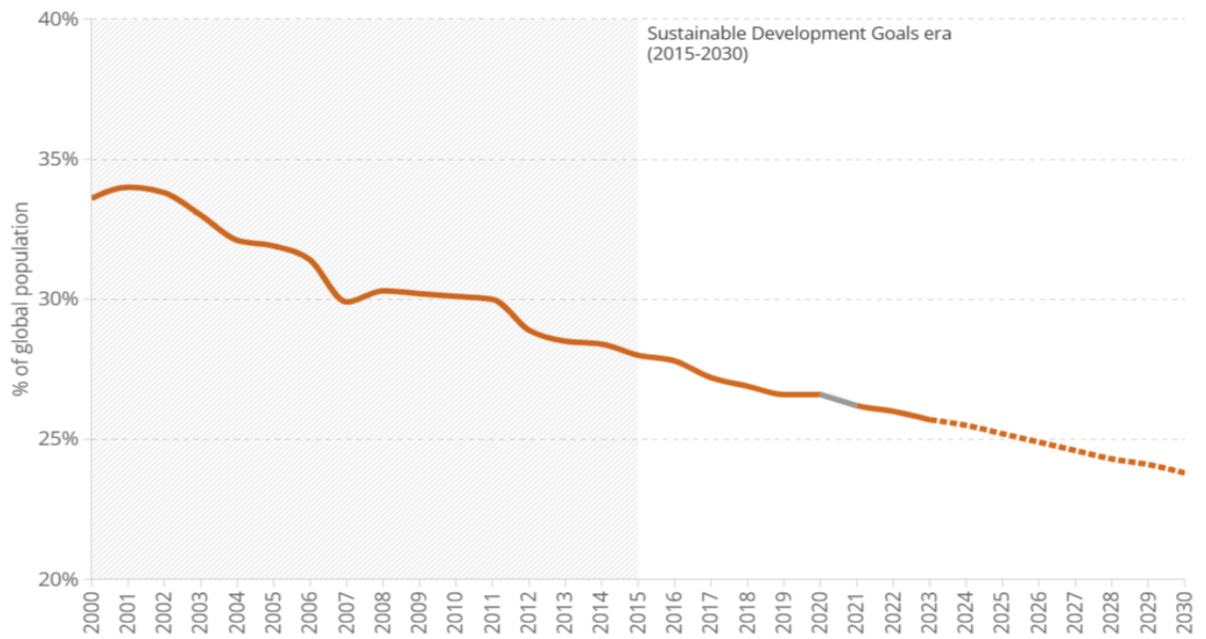


Figure 2. Population facing financial hardship because of healthcare costs.

(2025)



Helpful Resources:

Helpful Videos:

- https://www.youtube.com/watch?v=_htX-EdAyuc
- <https://www.youtube.com/watch?v=XM-X4fQHsW0>
- <https://www.youtube.com/watch?v=AmRXqaG3Igo>

Other Helpful Resources:

- <https://www.who.int/health-topics/health-equity>
- <https://www.who.int/publications/i/item/WHO-EURO-1978-3938-43697-61471>
- https://www.oecd.org/en/publications/2025/11/health-at-a-glance-2025_a894f72e/full-report/unmet-needs-for-healthcare_c994e8b1.html

- <https://www.who.int/news-room/fact-sheets/detail/universal-health-coverage-%28uhc%29>
- <https://www.gov.uk/government/publications/health-inequalities-in-health-protection-report/health-inequalities-in-health-protection-report-2025>
- <https://www.who.int/news-room/feature-stories/detail/resilience-in-india-s-tribal-heartland-to-achieve-health-for-all>